Phesgo + weekly paclitaxel

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Taxane + Phesgo phase

Active medicines: Paclitaxel + Pertuzumab + Trastuzumab

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before paclitaxel.
- Day 8 and thereafter: no routine dexamethasone; chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before. Consultant may add steroid for risk/prior reaction or omit famotidine if appropriate; monitor closely.
- Rechallenge steroid changes require consultant guidance; consider analgesia for taxane aches and granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00797 v4 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.15 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Phesgo-only maintenance

Active medicines: Pertuzumab + trastuzumab (Phesgo)

Emetogenic risk: Minimal for Phesgo / trastuzumab alone

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine prophylactic antiemetic or cytotoxic premedication is carried over. Previous reactions or symptoms need an individual plan.
- Phesgo premedication is not usually required; consider paracetamol/antihistamine for the individual reaction history.
- After chemotherapy stops, use the agreed Phesgo maintenance plan (National Cancer Control Programme (NCCP) 00785 when eligible). Keep cardiac and response checks; chemotherapy steroids, antiemetics and growth factors do not automatically continue.
- Continue cardiac monitoring; maintenance does not restart the echo clock.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00797 v4 · p6 · NCCP antiemetics v8, Table 1 pp4–5

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